From Law to Reality: Understanding Why Social Policies Fail During Implementation

1. Introduction

The adoption of social policies is usually regarded as an important symbol of social progress, but the recognition of rights in law does not necessarily translate into the protection that target groups can obtain in real life. When the policy enters the implementation stage, its effect will be affected by institutional resources, responsibility division, professional training, discretion of front-line executors, social and cultural concepts and other factors. Tummers and bekkers (2014) pointed out that front-line executives such as police, teachers, medical personnel and social workers do not implement policies mechanically, but make judgments based on their own understanding, working conditions and organizational environment. Therefore, there may be an obvious "implementation gap" between the policy text and the actual results.

The domestic violence protection policy reflects this contradiction. The mainland of China passed the anti domestic violence law of the people's Republic of China in 2015 and formally implemented it in 2016, establishing systems such as public security warning, personal safety protection order and mandatory report. Hong Kong mainly protects victims through injunctions and related social services in accordance with the domestic and cohabitation Violence Ordinance (Chapter 189). Both systems confirm that domestic violence is not only a domestic dispute, but also a social problem requiring the intervention of public institutions. However, whether legal rights can be translated into practical security still depends on whether the police can identify domestic violence, how the court judges evidence, whether medical and social welfare institutions can refer in time, and whether the victims are willing and able to enter these systems. A study of Chinese police officers found that their attitude towards domestic violence was closely related to

their tendency to take formal intervention measures (Zhao et al., 2018). Studies in mainland China, Hong Kong, Macao and Taiwan also showed that medical professionals' preparation for domestic violence identification, screening and referral was inconsistent (Li et al., 2020).

A similar gap between law and reality also appears in the menstrual health policy. The city of New York took the lead in requiring relevant public schools to provide free menstrual supplies in 2016, and the state of New York subsequently expanded the requirement to public schools serving students in grades 6 to 12 in 2018, and further expanded the coverage in 2024. The policy redefines menstrual supplies from personal consumer goods as the basic resources needed by students to participate in education. However, the purchase of products by the school does not necessarily mean that students can obtain products stably, conveniently and privately. The case study of New York City found that distributor installation, daily replenishment, equipment maintenance, students' knowledge and feedback channels all affect the actual accessibility of policies (Schmitt et al., 2022). At the same time, it is difficult to automatically eliminate menstrual stigma, lack of knowledge and toilet privacy by providing only sanitary products (Olson et al., 2022).

Most of the existing studies discussed domestic violence or menstrual health policies separately, and less compared the similar implementation mechanisms in different policy areas. This paper therefore studies: from 2015 to 2025, to what extent will the domestic violence protection policies in mainland China and Hong Kong, as well as the menstrual equity policy in New York, transform legal recognition into accessible and effective practical support? This paper uses a structured comparative case study to analyze the five links of policy design, institutional

capacity, front-line implementation, service accessibility and supervision and accountability through legal texts, government data and peer-reviewed literature. This paper argues that legal recognition is only the starting point of social change; When the responsibilities of institutions are unclear, resources are insufficient, professional training is limited, implementation is excessively dependent on personal discretion, or the target group is still affected by stigma and access barriers, formal rights are difficult to be transformed into stable and realistic protection.

2. Literature Review and Analytical Framework

2.1 Policy Implementation and the Implementation Gap

Early policy research often regarded legislation or policy formulation as the main outcome of government action, but the adoption of policy is not equal to the achievement of policy objectives. Policy implementation research therefore emphasizes that we should distinguish between policy output, policy results and policy impact. Policy output refers to the establishment of laws, systems, funds or service projects; Policy results focus on whether the target groups really get services; Policy impact is to examine whether the policy has improved its safety, health, education or social situation. Durlak and Dupre (2008) pointed out after reviewing more than 500 studies that the quality of implementation will significantly affect the project results, and if effective policies are not fully implemented, they may also have limited or even inconsistent effects.

"Implementation gap" can thus be understood as the gap between policy commitments and actual results. Matland (1995) believes that policy implementation is affected by the ambiguity of objectives and the degree of conflict of interest. When policy concepts are unclear, institutional responsibilities overlap, or different participants have different understandings of policy objectives,

local institutions and front-line personnel will have more room for interpretation. For example, the anti domestic violence policy may explicitly prohibit physical injury, but the identification of front-line personnel on psychological violence, economic control or compulsory control may still be different. In contrast, the legal objectives of the policy of free menstrual supplies are relatively specific, but "providing products in schools" may still not fully explain where products should be placed, who should supplement them, how to repair the dispenser, and how to protect students' privacy. Therefore, the implementation gap can be derived not only from the legal design, but also from the transformation process after the law enters the specific institutions.

2.2 Front line executors and discretion

Lipsky (2010) proposed the concept of "street bureaucrat", which is used to describe the staff who directly contact the public and are responsible for implementing public policies. These personnel include police, teachers, medical staff, social workers and school administrators. Due to the complexity of real cases, limited resources and the inability of policy rules to cover all situations, front-line executives must use discretion. Brodkin (2012) further pointed out that the front-line institutions are not passive endpoints in the policy transmission chain, but the place where the actual policy content is formed. The final policy that the public is exposed to is often the version after the front-line personnel explain and implement it.

Discretion does not necessarily lead to policy failure. It can help staff adjust services according to individual needs, but it may also make the policy results affected by personal attitudes, professional knowledge and organizational environment. May and winter (2009) found that management methods, organizational signals and the propensity of front-line personnel all affect

the implementation of policies. Tummers and bekkers (2014) also pointed out that the role of discretion depends on whether the executor understands and supports the policy and whether he has the ability to complete the task.

This theory is particularly applicable to domestic violence policies. The police need to determine whether the incident constitutes domestic violence, whether there is an immediate risk, and whether mediation, warning or formal measures should be taken. Chinese police research shows that police officers' understanding and attitude towards domestic violence is related to their tendency to take formal actions such as arrest (Zhao et al., 2018). In the menstrual health policy, the risk of discretion is usually not manifested in legal judgment, but in the daily management of the school. School personnel decide the specific location of supplies, replenishment frequency, troubleshooting and student notification methods. These seemingly small decisions will directly affect whether students can obtain supplies stably and privately (Schmitt et al., 2022).

2.3 Institutional capacity and inter agency coordination

Focusing only on personal attitudes is still not enough to explain policy implementation. The behavior of the executor is limited by the institutional capacity, including capital, manpower, training, operation guide, data system and organizational support. Nilsen (2015) pointed out that the implementation research needs to examine the policy intervention itself, the implementation process and the organizational environment at the same time, and can not simply attribute the unsatisfactory results to individuals' unwillingness to implement.

Domestic violence policies in particular rely on inter agency cooperation. Victims may be exposed to the police, courts, hospitals, social welfare institutions and asylum services in turn.

Even if each institution has performed some of its duties, victims may not be able to obtain continuous protection as long as the referral, information sharing or follow-up tracking is interrupted. A review of IPV nursing research in mainland China, Hong Kong, Macao and Taiwan shows that medical personnel's readiness in identification, screening and referral is not consistent (Li et al., 2020). This shows that training, standard process and inter agency cooperation are still needed after the rights are granted by law.

The institutional chain of menstrual health policy in New York is relatively short, but it also relies on administrative capacity. The government or education department needs to provide policy requirements. Schools need to purchase supplies. Facility personnel are responsible for installation, replenishment and maintenance. Teachers and administrative personnel need to pass information to students. If there is no stable budget, clear responsibilities and reporting procedures, the school may have distributors in form, but it can not guarantee the availability of products in the medium and long term.

2.4 Accessibility, privacy and social stigma

Policy implementation should also be evaluated from the perspective of target groups. The existence of services does not mean that services can be used safely, conveniently and with dignity. Victims of domestic violence may give up seeking help because of economic dependence, risk of retaliation, family pressure or fear of not being trusted. Even if medical or judicial institutions have established services, if the cost of entering the procedure is too high, formal rights may still be difficult to use.

Menstrual supplies may also be in a state of "available in form and difficult to obtain in practice". For example, if supplies are only stored in the nurse's office, students may need to publicly state their needs; Distributors installed in places where privacy is lacking may also reduce the willingness to use. Olson et al. (2022) pointed out that menstrual policies usually pay more attention to material issues such as supplies and facilities, but less directly deal with stigma and lack of menstrual knowledge. Stigma is not only the social background outside the implementation of the policy, but also affects whether students use products, whether they report shortages, and whether schools are willing to openly discuss menstrual health. Therefore, accessibility should include material accessibility, information accessibility, privacy, security and social acceptance.

2.5 The analytical framework of this paper

Based on the above literature, this paper defines the "implementation gap" as the gap between the formal policy rights and the protection or support actually obtained by the target group, and compares two cases from five dimensions. First, the clarity of policy design and objectives, including whether the rights, applicable objects and institutional responsibilities are clear. Second, institutional capacity, including capital, manpower, training, equipment and inter agency coordination. Third, front-line implementation, including staff's understanding, attitude, discretion and operation methods of policies. Fourth, service accessibility, including information, location, privacy, security and social stigma. Fifth, supervision and accountability, including implementation data, complaint channels, user feedback and correction mechanisms.

These five factors are not independent of each other. Unclear responsibilities may increase front-line discretion, insufficient resources may reduce the quality of training and service, and

insufficient supervision will make inconsistent implementation exist for a long time. Social stigma may reduce help seeking, use and complaints, making it difficult for the system to find real problems. Therefore, this paper expects that when multiple obstacles appear in the policy implementation chain at the same time, the gap between legal recognition and actual protection will be more obvious. Through this unified framework, this paper is not to judge which of the domestic violence policy and the menstrual health policy is "more successful", but to compare how the two types of social policies with obvious differences produce similar or different implementation obstacles in the process of entering daily life from the legal text.

3. Methodology

3.1 Research Design

This paper uses the method of qualitative comparative case study to analyze the gap between the implementation of domestic violence protection policy and school menstrual health policy from 2015 to 2025. The case study is suitable for analyzing the complex relationship between the law, executive agencies, front-line personnel and target groups, and can put the policy back into the specific social and institutional environment for understanding (Yin, 2018). This paper further adopts the "structured and focused comparison" proposed by George and Bennett (2005): each case uses the same analysis questions, but only focuses on the evidence directly related to the implementation of the policy, and does not comprehensively discuss all the causes of domestic violence or menstrual health problems.

This paper sets up two main cases. The first case study is the implementation of the domestic violence law in mainland China after its implementation in 2016, and the operation of

the domestic and cohabitation Violence Ordinance in Hong Kong from 2015 to 2025 is taken as a regional reference. Because Hong Kong and the mainland have different legal systems, this paper will not combine the data of the two places, but compare whether there are similar frontline identification, professional training, service referral and social stigma problems in the two places. The second case study is the development and implementation of the menstrual equity policy in New York, including the city's leading policy in 2016, the school free menstrual supplies regulation in New York State in 2018, and the coverage expansion in 2024. The policy areas and institutional environments of the two cases are obviously different, but both try to transform the needs that were previously regarded as private issues into the responsibilities of public institutions, so they are suitable for identifying cross domain joint implementation mechanisms.

3.2 Data Collection and Source Selection

This paper uses second-hand data and does not carry out questionnaires, interviews or experiments. The information is mainly collected through Scite academic database and official websites of government and legislative bodies. The search keywords include "domestic violence law" "policy implementation" "protection order" "policy dispersion" "menstrual equity" "free period products" "school implementation", combined with regional vocabulary such as "China" "Hong Kong" "New York". The included data must be published or related to the implementation of policies from 2015 to 2025, and can provide information on legal content, implementation procedures, professional behavior, service accessibility or policy results.

The data sources are divided into four categories: the first category is legal texts and government documents, which are used to confirm policy requirements and institutional

responsibilities; The second category is peer-reviewed papers, which are used to analyze the evidence of policy implementation; The third category is statistics, guidelines and assessments issued by courts, education departments and public institutions; The fourth category is the reports issued by IPV research, trpi and other social organizations to supplement the actual experience of the target group. Social organizations and advocacy agencies may have specific policy positions, so only when the research methods, data sources and sample information are transparent, relevant materials will be used to support the main argument. The news report is only used to explain the policy background and is not the main basis for the core conclusion. Document analysis can integrate different types of data, but researchers still need to assess the purpose, credibility and background of the document (Bowen, 2009).

3.3 Data Analysis

All data were coded according to the five dimensions established by the literature review: policy design and goal clarity, institutional responsibility and resources, front-line training and discretion, service accessibility and stigma, and supervision accountability and user feedback. The analysis starts with rebuilding the policy implementation chain within each case. For example, the investigation of domestic violence cases covers the process from violence identification, alarm, risk assessment to protection orders and service referral; The New York case examines the process from legislation, procurement and product installation to replenishment, students' knowledge and actual use. Then, this paper compares the interruption of the two cases in which links, and distinguishes the common mechanism and case specific factors.

This article will not judge the success only by the existence of the policy, nor will it identify the complete failure of the policy because the social problems still exist. The evaluation will distinguish policy outputs, policy outcomes and policy impacts: the adoption of laws or the installation of equipment are policy outputs; The access of target groups to services is the result of policy; The improvement of safety, health or education is the policy impact. Through this method, this paper aims to determine the link where the implementation gap appears and its possible reasons, rather than simply ranking the two policies.

3.4 Reliability, Ethics and Limitations

In order to improve the credibility of the research, this paper will try to use the legal text, government data and peer review research for cross validation, and record the source, year, region and research method of each evidence. If different data come to the opposite conclusion, this paper will retain the differences and analyze the possible reasons, rather than only choose the evidence to support the presupposed argument. Because the research only uses public documents and does not involve the direct recruitment of research participants, the ethical risk is relatively limited; However, when quoting the experiences of victims of domestic violence, they will still avoid presenting identifiable information or unnecessary injury details.

The main limitation of this study is that the policy objectives and risk levels are different between cases, and the statistical caliber of secondary data is not completely consistent. In addition, the increase in reported cases may represent an increase in violence or an improvement in recourse channels; The purchase of products by schools does not necessarily mean that students can obtain products continuously. Therefore, this paper mainly identifies the implementation mechanism

supported by evidence, rather than claiming that there is a strict direct causal relationship between the two cases.

4. Case 1: Domestic Violence Protection Policies in Mainland China and Hong Kong

4.1 Legal design of anti domestic violence policy in mainland China

In December, 2015, China passed the law of the people's Republic of China on combating domestic violence, which was formally implemented on March 1st, 2016. This is China's first national law specifically targeting domestic violence, marking that domestic violence has changed from a problem usually regarded as an internal contradiction in the family to a public affair that needs to be handled jointly by public security, judicial, medical, educational and social welfare institutions. The law defines domestic violence as physical or mental infringement between family members by beating, binding, maiming, restricting personal freedom and frequent abuse and intimidation, and establishes mechanisms such as public security warning, mandatory reporting, personal safety protection orders and temporary asylum (Standing Committee of the National People's Congress, 2015).

From the perspective of legal design, victims can enter the protection system through multiple paths. After receiving the report, the public security organ shall stop domestic violence, investigate and collect evidence, and issue a warning letter to the perpetrator when the behavior has not yet met the conditions for public security administration punishment; The court may issue a personal safety protection order according to the application to prohibit the perpetrator from continuing to commit violence, harassment or contact with the victim; Schools, hospitals and social welfare institutions also bear the responsibility of compulsory reporting when they find that

persons without or with limited civil capacity suffer from domestic violence. These Provisions show that the law is not only concerned with punishment after the event, but also attempts to establish prevention, intervention and continuous protection mechanisms.

However, in order to produce practical protection, this system requires continuous action by multiple institutions. The ideal implementation path should include: victims or third parties identify and report violence, public security organs complete risk judgment and evidence records, courts apply for protection orders based on material review, medical and social service institutions provide treatment, asylum and referral, and finally relevant institutions supervise the implementation of protection measures. As long as one link is interrupted, formal rights may not be translated into real security.

4.2 Police intervention and frontline discretion

The public security organ is usually the first official entrance to the victim contact protection system, so how police officers understand domestic violence will directly affect the follow-up procedure. Although the law stipulates that domestic violence includes physical and mental abuse, the violence in real cases is often continuous and may include economic control, threat, isolation and emotional manipulation. These behaviors may not leave obvious scars, and it is difficult to fully present through an alarm. When police officers mainly rely on visible injuries to determine the seriousness of the case, continuous control may be understood as a general family conflict.

Zhao et al. (2018) conducted a situational survey of 520 police officers in a city in southern China and found that the police officers' attitude was closely related to their tendency to take formal

action in domestic violence cases. This means that even if the laws are the same across the country, different police officers may handle similar cases differently. Mediation may ease the immediate conflict in some cases, but if there is obvious power inequality in the violent relationship, excessive reliance on mediation may also ignore the continuous risk of victims.

Organizational environment also affects individual discretion. Zhao et al. (2024) studied community and criminal justice professionals and found that there was a correlation between the legal support and organizational support perceived by practitioners and their choice of mediation or formal intervention. If the organization does not provide clear process, professional training or management support, front-line personnel are more likely to rely on personal experience to handle cases. Therefore, the difference in implementation cannot be entirely attributed to the attitude of individual police officers, but should be investigated whether the police agencies provide risk assessment tools, evidence recording standards and continuous training.

4.3 Personal safety protection order and judicial determination

The personal safety protection order enables victims to seek immediate protection beyond divorce proceedings or criminal judgments, which is an important institutional innovation of the anti domestic violence law. However, whether the protection order can work depends on whether the victim knows the application channel, whether he can submit supporting materials, and how the court determines the domestic violence and real danger.

Zheng et al. (2025) pointed out that the identification of domestic violence still faces difficulties in the application of evidence and definition through the analysis of the judicial practice of personal safety protection orders in China. Physical injury can be proved by photos, medical

records or alarm records, but psychological violence, threats and long-term control usually lack single and direct evidence. If judicial review excessively relies on traditional written evidence, the victims who are most difficult to leave records may be the most difficult to obtain protection. In addition, the victim's failure to report to the police does not necessarily mean that the violence does not exist, but can also result from fear, economic dependence or lack of trust in the system.

Therefore, the number of protection orders or the proportion of approval cannot independently indicate whether the law is successful. Fewer applications may mean less violence, or insufficient awareness of the system; The increase in applications may reflect the deterioration of the problem or the improvement of the channels for help. When evaluating the implementation effect, it is necessary to investigate the application channel, trial time, evidence standard, approval and implementation after the issuance of the protection order.

4.4 Medical care, social services and victim accessibility

The consequences of domestic violence are not limited to the judicial field. Victims may also be found first in hospitals, schools or social service institutions. Li et al. (2020) reviewed the IPV nursing research in mainland China, Hong Kong, Macao and Taiwan, and found that there were differences in the recognition and response of medical staff to IPV in different regions. Medical staff have the conditions to detect repeated injuries, provide confidential support and referral services, but if they lack screening knowledge, referral process or safe communication environment, the medical system may miss the opportunity of intervention.

Whether the service exists or not does not mean that the victim can use it. Help seeking may be accompanied by economic losses, housing difficulties, child care problems and the risk of

retaliation by the perpetrator. The research on married immigrant women across the mainland and Hong Kong shows that immigration status, social support, family power and economic dependence may interact, making it more difficult for some victims to leave violent relationships or obtain continuous support (Chiu, 2017). This shows that policy accessibility depends not only on whether the law allows the application for protection, but also on whether the victim can enter the system without greater risk.

4.5 Legal mechanism and implementation environment in Hong Kong

Hong Kong mainly provides legal protection under the family and cohabitation Violence Ordinance (Chapter 189). The regulation allows eligible victims to apply to the district court for an injunction. The court can prohibit the perpetrator from harassing the applicant, exclude the perpetrator from the common residence, and attach an arrest authorization (Hong Kong e-legislation, n.d.) when the conditions are met. Compared with the mainland's anti domestic violence law, the Hong Kong system focuses more on the coordination of the civil injunction and the existing police, medical and social welfare systems, rather than uniformly stipulate the prevention and intervention responsibilities of all institutions through a comprehensive law.

There may also be a gap between legal remedies and actual use in Hong Kong. Victims need to identify their experiences as domestic violence, understand the application channels, and have the conditions to seek legal and social services. Leung (2019)'s research on Hong Kong's news reports found that media discourse may still repeat the social myth about IPV and adversely affect the victims' help seeking. When violence is described as a private conflict, or the victim is

questioned why he did not leave the relationship in time, social stigma may reduce the willingness to report, and may also affect the way front-line personnel understand the case.

The legal systems of Hong Kong and the mainland are different, but both show similar implementation challenges: non physical violence is difficult to identify; Formal procedures rely on evidence and professional judgment; The need for effective referral between police, medical, judicial and social services; Economic dependence, family values and stigma may prevent victims from seeking help. However, the differences between the two places cannot be ignored. Mainland cases are more suitable to investigate how comprehensive laws are enforced by local public security and courts, while Hong Kong cases are more suitable to analyze whether civil injunctions and existing service networks can form continuous protection.

4.6 Phased conclusion

The experience of mainland China and Hong Kong shows that legal recognition does provide new protection language and formal relief channels for victims, but the implementation effect depends on the complete system chain. The actual effect of the law is determined by whether the police can identify the risk, how the court handles the evidence, whether the medical staff can find and refer, and whether the victims can enter the system safely. The main problem of domestic violence policy is not simply "failure to abide by the law", but that formal rights may be reduced, delayed or interrupted after multi-level institutions and professional judgment. It can be seen that reducing the implementation gap not only needs to improve the legal provisions, but also needs unified procedures, professional training, inter agency coordination and victim safety centered service design.

5. Case 2: New York Menstrual Equity Policy

5.1 Policy background and development process

The shortage of menstrual supplies is not only a problem of personal consumption ability, but also may affect students' health, classroom participation and education opportunities. If students cannot get sanitary napkins or tampons in time, they may need to leave school, miss classes, or use unsafe substitutes. Therefore, the menstrual equity initiative advocates that basic menstrual supplies should be regarded as school hygiene necessities similar to toilet paper and hand sanitizer, rather than private consumer goods that must be prepared by students themselves. Francis et al. (2023) pointed out that the development of school menstrual health policy reflects the link between public health and education equity, but there are still significant differences in coverage objects, product locations, funds and implementation responsibilities in different states.

New York City is one of the regions in the United States that adopted the policy of free menstrual supplies for schools earlier. Around 2015, New York City first carried out pilot projects in some public schools to understand students' needs and test the feasibility of installing free supplies dispensers in toilets. In 2016, the New York City Council passed proposal 1128-a, requiring the Municipal Department of education to provide free menstrual supplies in the toilet in the relevant school buildings serving girls in grades 6 to 12 (New York City Council, 2016). Compared with the past way of going to the nurse's office to get supplies, placing the products in the toilet can reduce the pressure of students' explaining their personal needs, and is more conducive to protecting privacy.

In 2018, the state of New York passed article 267 of the public health act, extending the requirement for free menstrual supplies to public schools serving students in grades 6 to 12 throughout the state. In 2024, the scope of the policy will be further extended to relevant non-public schools, while allowing non-public schools that do not serve students with menstrual needs to apply for exemption when they meet certain conditions (New York state legislation, 2024). This development process shows that New York's policy has gradually transformed from a local pilot to a state-level system, and the acquisition of menstrual supplies has changed from the welfare provided by the school voluntarily to the basic support that students should enjoy in the school environment.

5.2 From legal requirements to daily school implementation

The formal right established by the New York policy is relatively clear: schools should provide menstrual supplies free of charge in the toilet. However, to realize this right in daily life, it still needs to go through many links, such as procurement, equipment installation, replenishment, maintenance, information dissemination and problem feedback. The ideal implementation path can be summarized as follows: the state or city makes laws, the education department explains the policy requirements, the school purchases supplies and distributes equipment, the facility personnel are responsible for the installation and supplement, the students understand the location and free nature of supplies, and finally report the shortage or equipment failure through the feedback mechanism.

The case study of Schmitt et al. (2022) on the menstrual equity policy in New York City schools shows that policy implementation is not an automatic process after the completion of

legislation. In the pilot phase, policy makers first need to address the school's concerns about product waste, equipment damage and daily management. The important role of the pilot is not only to prove that students need products, but also to test whether the school can integrate this demand into the existing facilities and administrative system. Research shows that putting the product directly in the toilet is a student-centered design, because students do not need to make a request to nurses or teachers, and can handle menstruation more timely and privately.

However, the entry of products into schools does not mean that the implementation of the policy has been completed. School facilities and cleaning personnel play a key role in installing distributors, replenishing products, inspecting equipment, and submitting replenishment orders (Schmitt et al., 2022). This shows that the actual implementation of the policy largely depends on the staff who are not usually regarded as policy implementers. The principal and administrative staff mainly play the role of communication and coordination, and whether the product is continuously available depends on whether the daily maintenance responsibilities are clear. If there is no designated person to check the dispenser, or the replenishment process is too slow, the school may have equipment in form, but it can not guarantee that students can get the product when they need it.

5.3 Students' knowledge, privacy and practical accessibility

Practical accessibility not only requires the existence of the product, but also requires students to know the location of the product, understand its free nature, and be able to use it without shame. Schmitt et al. (2022) pointed out that explaining the new policy to students is an important link in the implementation process. If the school only installs dispensers without notification, some

students may still think that supplies need to be paid, or continue to go to the nurse's office for help. In addition, students are more likely to report shortage or equipment failure to the school only when they know that they have the right to free supplies.

The placement of products also affects the policy effect. The direct provision of products in the toilet can shorten the time for students to leave the classroom and reduce the embarrassment of openly explaining menstrual needs to adults. However, toilet door locks, garbage disposal facilities, cleanliness and the location of the dispenser will also affect the use experience. If the product only appears in some toilets, or the relevant toilets are not easy to access, the legal free supply may still form a practical obstacle. Studies in other parts of the United States have also found that there is a link between the lack of menstrual supplies and students' absence from work and the use of school resources to seek products, indicating that the access channels within schools have practical significance for educational participation (KUHLMANN et al., 2024).

The early New York policy mainly used gender language such as "feminine hygiene products" and focused on serving "female students". This expression has raised the attention to the needs of girls, but it may also ignore the trans gender or non binary students with menstrual needs. Subsequent policies gradually use more relatively inclusive expressions such as "menstrual products". It can be seen that the definition of the target group is not simply a matter of words, but will affect which toilets the products are placed in, which students are told to have the right to use, and how schools understand menstrual equity.

5.4 Scope restrictions of free supplies policy

Free supplies can reduce economic and temporary access barriers, but menstrual health is not just about products. Students may also face menstrual pain, lack of health knowledge, inappropriate toilet facilities, and ridicule or shame caused by menstruation. Olson et al. (2022) reviewed menstrual policies and found that many policies mainly emphasize material solutions such as supplies and facilities, and less directly deal with menstrual stigma. The policy can make the products easier to obtain, but it can not eliminate students' concerns about menstrual blood leakage, public collection or discussion of menstruation by relying on the dispenser alone.

Therefore, if we only count how many products the school has purchased or how many distributors it has installed, we may overestimate the effect of the policy. The policy evaluation should also examine whether students know the existence of supplies, whether distributors are often out of stock, whether product types meet the needs, whether students can use them privately, and whether they have reduced anxiety, left class or absent from work after obtaining supplies. Ballard et al. (2025) also pointed out that the systematic comparison of school menstrual health policies in various states in the United States also pointed out that it is not enough to reflect the quality of policies only by comparing whether the states have passed laws, but also need to pay attention to the coverage grade, product location, funding arrangements and implementation requirements.

5.5 Supervision, feedback and accountability

Another important issue of the New York policy is how to find daily failures in the implementation. Because the shortage or equipment damage usually occurs at the level of a single

school or toilet, it is difficult for the state legislature to directly grasp the actual situation. By the end of the research period, New York City public schools had explained their right to get free products to students, and provided feedback channels to report shortages and distributor problems (New York City public schools, n.d.). This indicates that the policy has shifted from one-way supply to allowing users to participate in supervision.

However, the feedback mechanism is only effective when students know the channel, believe that complaints will be handled and are not worried about revealing their personal identity. The school also needs to record the location, frequency, processing time and replenishment results of the problem. If the supervisor only checks whether the school is equipped with equipment, but does not check whether the equipment is in continuous operation, he can only measure the formal compliance and cannot evaluate the actual service quality. A more complete assessment should include input indicators, implementation indicators and outcome indicators, such as the number of products purchased, the normal operation rate of equipment, students' awareness, actual use experience and menstrual related absences.

5.6 Phased conclusion

The menstrual equity policy in New York has successfully redefined menstrual supplies from private responsibility as the basic resources that schools should provide, and gradually established formal rights through New York City pilot, state legislation and coverage expansion. However, the case also shows that the legal requirements must rely on a series of daily administrative actions to produce practical results. Problems in any link of procurement,

installation, replenishment, maintenance, publicity, privacy and feedback may make "free provision" stay at the formal level.

Therefore, the implementation gap in the New York case is not that the school publicly refuses to enforce the law, but that the formal rights are weakened in the daily management. Products may have been purchased but not supplemented in time; The distributor may have been installed, but students do not know its existence; Supplies may be free, but stigma and privacy issues still prevent students from using them. Real menstrual equity not only requires products to enter school, but also requires students to obtain products continuously, conveniently and with dignity, and receive corresponding health education and system support.

6. Comparative Discussion

6.1 From private issues to public responsibility

Domestic violence protection policy and school menstrual health policy belong to different policy areas. The former involves personal safety, judicial relief and national coercive force, while the latter mainly involves public health, school management and educational equity. However, the two cases have similar policy changes: domestic violence is no longer completely regarded as an internal family affair, and menstrual supplies are no longer simply regarded as consumer goods that individuals should bear on their own. Both types of policies redefine government responsibilities through laws and give specific groups formal rights to protection or resources.

Such legal recognition is of great significance. The anti domestic violence law of the mainland of China has established mechanisms such as public security warning, personal safety protection order and mandatory reporting, while the Hong Kong family and cohabitation Violence

Ordinance provides civil remedies such as injunctions. The menstrual equity policy of New York City and New York state requires schools to provide free menstrual supplies in the toilet. They have changed the legal language that victims and students can use when making demands, respectively. However, the comparison results show that writing a need into the law is only the starting point of the policy implementation chain. Formal rights must also be translated into clear job responsibilities, operating procedures, service entrance and supervision mechanism before they can enter the daily life of the target group.

6.2 Common implementation gap

The first common problem of the two cases is that there is room for transformation between legal provisions and institutional implementation. Domestic violence policy needs to be implemented jointly by public security, courts, medical and social service institutions, but different institutions pay attention to different goals. The police may pay attention to whether the immediate conflict has stopped. The court pays attention to whether the application materials meet the certification requirements. The medical staff mainly deal with physical injuries. If there is no continuous referral and information connection, even if each institution completes local tasks, the victims may still not be able to obtain complete protection.

The institutional chain of menstrual policy in New York is relatively short, but there is also the problem of responsibility transformation. The law stipulates that schools should provide products free of charge, but it may not specify who should inspect the dispenser, how often to replenish it, how to report equipment damage, and how to notify students. Schmitt et al. (2022) found that school facilities and cleaning personnel actually undertake key tasks such as installation,

maintenance and replenishment. This shows that the effect of the policy may depend on the administrative positions that are inconspicuous in the legal text but indispensable in the daily implementation.

The second common problem is frontline discretion. Lipsky (2010) pointed out that front-line staff need to explain policies under the conditions of incomplete rules and limited resources. The discretion in domestic violence cases mainly shows how the police judge the danger, how the court determines the evidence, and whether the medical staff actively identify and refer. Zhao et al. (2018) showed that the attitude of police officers towards domestic violence was related to the tendency of formal intervention. Although discretion in New York schools does not involve the same degree of coercive power, it will affect product placement, replenishment, information dissemination and complaint handling. The severity of the two kinds of discretion is different, but both may cause the same policy to produce inconsistent results in different places.

The third common issue is the actual accessibility of services. Victims of domestic violence may avoid seeking help because of economic dependence, risk of retaliation, family pressure or fear of not being trusted. Students may not use the dispenser because of shame, lack of privacy or not knowing that the product is free. Therefore, there is a situation that "the service already exists, but the target group is unable or unwilling to use" in both types of policies. Stigma is not the cultural background outside the policy implementation, but an important variable that directly affects the policy entrance. Leung (2019) pointed out that the myth of domestic violence in Hong Kong media may not be conducive to victims' help seeking; Olson et al. (2022) also found that many menstrual policies emphasize material supply, but less directly deal with menstrual stigma.

6.3 Differences between the two types of policy implementation mechanisms

Despite the existence of a common structure, the implementation barriers of the two cases cannot be completely equivalent. First, the domestic violence policy faces more complicated facts and legal judgments. "Domestic violence" may include physical injury, intimidation, psychological control and economic restrictions at the same time, but the evidence left by different forms is not the same. Zheng et al. (2025) pointed out that the practice of personal safety protection orders in China still faces difficulties in identifying domestic violence and judging evidence. In contrast, the basic requirements of the menstrual supplies policy are easier to observe: whether the school installs the dispenser, whether the product is provided and whether the product is free. However, clear legal objectives do not mean simple implementation, because continuous replenishment, privacy and students' knowledge are still difficult to be confirmed through only one inspection.

Secondly, the risk degree and time requirements of the two types of policies are different. Cases of domestic violence may involve sustained injury or even life-threatening. Wrong judgment or delayed referral may cause irreversible consequences. Therefore, domestic violence policy relies more on timely risk assessment, inter agency cooperation and continuous protection. Menstrual policies usually do not involve the same degree of immediate danger, but repeated shortages or inconvenient access can have long-term cumulative effects, including anxiety, class interruptions, absenteeism and health risks. Therefore, the former emphasizes timeliness and security, while the latter emphasizes stability, universality and daily availability.

Thirdly, the power relations of the two kinds of policies are different. The help of victims of domestic violence may be monitored by the perpetrators, and entering the system itself may

increase the risk; Students receiving menstrual supplies usually do not face the same form of direct retaliation, but may face peer ridicule, public identity and school gender norms. The accessibility of the former requires security first, while the latter focuses more on privacy, convenience and dignity. Therefore, both types of policies need to be user-centered, but the specific design cannot be exactly the same.

6.4 Formal compliance and substantive implementation

After comparing the two cases, it can be found that the implementation gap is not necessarily manifested by the public refusal of the institution to implement the law, and the more common form is "formal compliance". The police may complete the call out, but they do not fully record the continuous control or complete the service referral; The court may provide a channel for applying for a protection order, but it is still difficult for the victim to prepare supporting materials. The school may have purchased products and installed distributors, but it has not established stable inspection, replenishment and feedback procedures. Judging from the administrative records, these institutions may have completed the policy requirements, but judging from the experience of the target group, the rights are still not reliable enough.

Therefore, policy evaluation cannot only use output indicators that are easy to count. The number of police calls, the number of protection order applications, the number of product purchases and the number of distributors can indicate that the government has taken action, but it can not independently prove that the target group has been effectively protected. The domestic violence policy also needs to investigate the processing time, the implementation of protection orders, the continuity of referral and the safety of victims; Menstrual policy needs to examine the

equipment operation, shortage frequency, students' awareness, privacy and actual use experience. Durlak and Dupre (2008) pointed out that the implementation quality will affect the policy or project results, which means that the same law may have different effects due to different local implementation capabilities.

6.5 Cumulative implementation gap

What the two cases reveal together is not a single obstacle, but a cumulative mechanism. First of all, the law or the implementation guidelines do not fully explain the organization and job responsibilities, resulting in a large space for front-line personnel to explain. Second, the lack of training, resources or organizational support makes discretion more vulnerable to personal attitudes and existing work habits. Thirdly, stigma, privacy and security barriers reduce help seeking, use and complaints, making it difficult for institutions to find problems. Finally, if the monitoring data only record the formal output, policy makers will not be able to adjust the system according to the actual experience of the target group.

These factors will reinforce each other. For example, if the victim of domestic violence does not report to the police for fear of not being trusted, the institution will lack records and may underestimate the service demand; If students fail to give feedback after finding that the dispenser is out of stock, the school may think that the existing replenishment system operates normally. Low usage or low reporting can not be simply explained as less demand, or it may be the result of insufficient accessibility. When the system allocates resources according to these incomplete data, the implementation gap will continue to exist.

6.6 Answers to research questions

The comparative results show that the failure of social progress policy to fully protect the target groups is not necessarily due to the wrong legal objectives, nor can it only be attributed to the lack of funds. A more complete explanation is that the law creates formal rights, but does not always establish an implementation chain that stably connects institutional responsibilities, front-line actions and the use experience of target groups. The main risks in the domestic violence policy focus on identification, evidence, professional judgment and inter agency protection; The main risks in New York's menstrual policy focus on procurement, maintenance, students' knowledge, privacy and continuous supply. But both show that the success of policies should be evaluated according to whether rights can be used safely, stably and with dignity.

Therefore, legal recognition is a necessary condition for social change, but not a sufficient condition. Only when the policy has clear responsibilities, sufficient resources, professional training, user centered service entrance and monitoring mechanism that can reflect the actual experience, the progress commitment in the law is more likely to be translated into practical protection.

References

The Standing Committee of the National People's Congress. (2015). Law of the people's Republic of China on combating domestic violence. https://www.scio.gov.cn/xwfb/gwyxwbgsxwfbh/wqfbh_2284/2021n_2711/2021n09y27rxw1/wjxgzc_4038/202208/t20220808_314382.html

Ballard, A. M., Wallace, E., Kaza, P., Self-Brown, S., Freeman, M. C., & Caruso, B. A. (2025). Mapping menstrual health and hygiene progress in US schools: A systematic policy review and comparison across states. *Frontiers in Reproductive Health, 7*, 1589772. https://doi.org/10.3389/frph.2025.1589772

Bowen, G. A. (2009). Document analysis as a qualitative research method. *Qualitative Research Journal, 9*(2), 27–40. https://doi.org/10.3316/QRJ0902027

Brodkin, E. Z. (2012). Reflections on street-level bureaucracy: Past, present, and future. *Public Administration Review, 72*(6), 940–949. https://doi.org/10.1111/j.1540-6210.2012.02657.x

Chiu, T. Y. (2017). Marriage migration as a multifaceted system: The intersectionality of intimate partner violence in cross-border marriages. *Violence Against Women, 23*(11), 1293–1313. https://doi.org/10.1177/1077801216659940

Durlak, J. A., & DuPre, E. P. (2008). Implementation matters: A review of research on the influence of implementation on program outcomes and the factors affecting implementation. *American Journal of Community Psychology, 41*(3–4), 327–350. https://doi.org/10.1007/s10464-008-9165-0

Francis, L., Meraj, S., Konduru, D., & Perrin, E. M. (2023). An update on state legislation supporting menstrual hygiene products in US schools: A legislative review, policy report, and recommendations for school nurse leadership. *The Journal of School Nursing, 39*(6), 536–541. https://doi.org/10.1177/10598405221131012

George, A. L., & Bennett, A. (2005). *Case studies and theory development in the social sciences*. MIT Press.

Hong Kong e-Legislation. (n.d.). *Domestic and Cohabitation Relationships Violence Ordinance (Cap. 189)*. https://www.elegislation.gov.hk/hk/cap189

Kuhlmann, A. S., Teni, M. T., Key, R., & Billingsley, C. (2024). Period product insecurity, school absenteeism, and use of school resources to obtain period products among high school students in St. Louis, Missouri. *The Journal of School Nursing, 40*(3), 329–335. https://doi.org/10.1177/10598405211069601

Leung, L. C. (2019). Deconstructing the myths about intimate partner violence: A critical discourse analysis of news reporting in Hong Kong. *Journal of Interpersonal Violence, 34*(11), 2227–2245. https://doi.org/10.1177/0886260516660298

Li, Q., Liu, H., Chou, K.-R., Lin, C.-C., Van, I.-K., Davidson, P. M., & Campbell, J. C. (2020). Nursing research on intimate partner violence in China: A scoping review. *The Lancet Regional Health—Western Pacific, 2*, 100017. https://doi.org/10.1016/j.lanwpc.2020.100017

Lipsky, M. (2010). *Street-level bureaucracy: Dilemmas of the individual in public services* (30th anniversary expanded ed.). Russell Sage Foundation.

Matland, R. E. (1995). Synthesizing the implementation literature: The ambiguity-conflict model of policy implementation. *Journal of Public Administration Research and Theory, 5*(2), 145–174.

May, P. J., & Winter, S. C. (2009). Politicians, managers, and street-level bureaucrats: Influences on policy implementation. *Journal of Public Administration Research and Theory, 19*(3), 453–476. https://doi.org/10.1093/jopart/mum030

New York City Council. (2016). *Int. No. 1128-A: Provision of feminine hygiene products in schools*. https://legistar.council.nyc.gov/LegislationDetail.aspx?GUID=834E4DFC-7F14-4E1E-812F-2CD862A4FC1D&ID=2637114

New York City Public Schools. (n.d.). *Menstrual products for students*. https://www.schools.nyc.gov/school-life/health-and-wellness/menstrual-products-for-students

New York State Legislature. (2018). *New York Public Health Law § 267: Menstrual products in schools* (original enactment). https://www.nysenate.gov/legislation/laws/PBH/267

New York State Legislature. (2024). *New York Public Health Law § 267: Menstrual products in schools* (version effective July 1, 2024). https://www.nysenate.gov/legislation/laws/PBH/267

Nilsen, P. (2015). Making sense of implementation theories, models and frameworks. *Implementation Science, 10*, Article 53. https://doi.org/10.1186/s13012-015-0242-0

Olson, M. M., Alhelou, N., Kavattur, P. S., Rountree, L., & Winkler, I. T. (2022). The persistent power of stigma: A critical review of policy initiatives to break the menstrual silence and

advance menstrual literacy. *PLOS Global Public Health, 2*(7), e0000070. https://doi.org/10.1371/journal.pgph.0000070

Schmitt, M. L., Booth, K., & Sommer, M. (2022). A policy for addressing menstrual equity in schools: A case study from New York City, U.S.A. *Frontiers in Reproductive Health, 3*, 725805. https://doi.org/10.3389/frph.2021.725805

Tummers, L., & Bekkers, V. (2014). Policy implementation, street-level bureaucracy, and the importance of discretion. *Public Management Review, 16*(4), 527–547. https://doi.org/10.1080/14719037.2013.841978

Yin, R. K. (2018). *Case study research and applications: Design and methods* (6th ed.). SAGE Publications.

Zhao, R., Zhang, H., Jiang, Y., & Yao, X. (2018). The tendency to make arrests in domestic violence: Perceptions from police officers in China. *International Journal of Offender Therapy and Comparative Criminology, 62*(16), 4923–4941. https://doi.org/10.1177/0306624X18801653

Zhao, R., Zhang, H., Wretman, C. J., Radtke, S. R., Rizo, C. F., Kim, J., & Macy, R. J. (2024). Exploring perceived legal and organizational support and attitudes on the decision to mediate or arrest in domestic violence cases: A study among community and criminal justice professionals in China. *Violence Against Women, 30*(14), 3558–3580. https://doi.org/10.1177/10778012231182411

Zheng, R., Xie, Z., & Duan, Z. (2025). Challenges identifying domestic violence in Chinese judicial practice: An empirical analysis of personal safety protection orders. *Violence Against Women, 31*(15–16), 3886–3909. https://doi.org/10.1177/10778012241297253